DSM-5-TR quick reference

Plain-words summaries written for exam review. Criteria are paraphrased, not quoted; check the DSM-5-TR (APA, 2022) for full criteria.

Rule out first

1. **Safety or emergency?** Check first for suicide or violence risk, overdose, delirium or dangerous withdrawal. Imminent risk or a medical emergency comes before any diagnosis.
2. **Substance or medication?** Ask about alcohol, drugs, prescriptions, over-the-counter products and recent changes. Symptoms that start with use, intoxication or withdrawal point to a substance/medication-induced disorder.
3. **Another medical condition?** Refer for medical evaluation when you see red flags: sudden onset, first episode after about 40, visual or tactile hallucinations, fluctuating attention, head injury or a new medication. Thyroid disease, infection, B12 deficiency and seizures are common mimics.
4. **Which mental disorder fits best?** Match duration and pattern, such as whether psychosis occurs only during mood episodes. Residual categories such as adjustment disorder come last.
5. **Normal, expected or cultural?** Grief, a proportionate stress reaction, or a belief or practice accepted in the client's culture may need no diagnosis. Without clinically significant distress or impairment, record context with a Z code if useful.

Durations at a glance

1 day
- Brief psychotic disorder: at least 1 day
- DMDD ruled out by manic or hypomanic symptoms over 1 day

3 days
- Acute stress disorder: 3 days to 1 month

4 days
- Hypomania: 4 days

1 week
- Mania: 1 week (any length if hospitalized)

2 weeks
- Schizoaffective: 2 weeks of psychosis without mood episode
- Major depressive episode: 2 weeks

1 month
- Brief psychotic disorder: under 1 month, full recovery
- Schizophreniform: at least 1 month (under 6)
- Schizophrenia: 1 month of active symptoms
- Delusional disorder: 1 month
- PTSD: more than 1 month
- Panic disorder: 1 month of worry or change

3 months
- Adjustment: onset within 3 months
- Bulimia: weekly for 3 months
- Insomnia: 3 nights a week for 3 months
- SUD early remission: 3 months
- IED (verbal or nondamaging): twice weekly for 3 months

6 months
- Schizophrenia: 6 months total
- Prolonged grief (youth): 6 months since death
- PTSD delayed expression: 6 months or later
- Adjustment: ends within 6 months after stressor ends
- GAD: 6 months
- Illness anxiety: 6 months

12 months
- PDD or cyclothymia (youth): 1 year
- Rapid cycling: 4 episodes in 12 months
- Prolonged grief (adult): 12 months since death
- SUD sustained remission: 12 months
- SUD criteria window: 12 months
- Personality disorder under 18: 1 year (never ASPD)

2 years
- Persistent depressive disorder (adult): 2 years
- Cyclothymic disorder (adult): 2 years

General rules

- Almost every diagnosis requires clinically significant distress or impairment; symptoms alone are not enough.
- Before naming a mental disorder, rule out the effects of a substance or medication and of another medical condition.
- Use 'other specified' when you state why criteria are not met, and 'unspecified' when you cannot or choose not to give a reason, such as in an ER with limited history.
- Add 'provisional' when you strongly expect a diagnosis but information or duration is still incomplete, as in schizophreniform disorder before 6 months.
- DSM-5-TR has no multiaxial system and no GAF; list diagnoses together, note medical conditions alongside, and the WHODAS 2.0 (Section III) can rate disability.
- Z codes record context such as housing, relational problems, maltreatment or a history of suicidal behavior; current suicidal behavior and self-injury have their own symptom codes; they are not mental disorders and never replace a risk assessment.
- List the principal diagnosis first: the main reason for the visit or admission.
- Judge symptoms against the client's cultural norms; the Cultural Formulation Interview (16 questions) can be used with any client, not only minoritized clients, ideally early in assessment.
- Do not diagnose a personality disorder from one session or during an acute episode of another disorder.
- A personality disorder can be diagnosed before 18 only if the traits

have lasted at least 1 year; antisocial personality disorder always requires age 18.
- A substance-induced diagnosis is doubtful if symptoms began before use or last about 1 month or more after withdrawal or intoxication ends; consider an independent disorder.

What changed in DSM-5-TR (2022)

- Prolonged grief disorder is new: at least 12 months after a death for adults (6 months for children and teens), with intense yearning or preoccupation for at least the last month.
- Unspecified mood disorder was restored for mood symptoms that cannot yet be sorted into a bipolar or depressive disorder, such as acute agitation in an ER with limited history.
- New symptom codes record current or past suicidal behavior and nonsuicidal self-injury; they are not mental disorders and can be used with or without a diagnosis.
- Stimulant-induced mild neurocognitive disorder was added.
- Intellectual disability was renamed intellectual developmental disorder; severity is still set by adaptive functioning, not IQ alone.
- Functional neurological symptom disorder is now the lead name, with conversion disorder kept in parentheses.
- Persistent depressive disorder dropped the word 'dysthymia' from its name; of the depressive-episode specifiers it keeps only anxious distress and atypical features, alongside pattern, onset, severity and remission specifiers.
- Bipolar I and II now state that episodes must not be better explained by schizoaffective disorder, add manic-episode severity definitions, and split psychotic-features specifiers by episode type.
- Autism spectrum disorder wording now makes clear that all three social-communication deficits are required.
- For PTSD in children 6 and younger, a redundant note on media exposure was removed; witnessing must be in person.
- Text was revised for racism and discrimination, using terms such as 'racialized' and 'ethnoracial' and addressing misdiagnosis risk in oppressed groups.
- Gender terms changed to 'experienced gender', 'gender-affirming' and 'assigned male or female at birth'.
- 'Neuroleptic' was replaced with antipsychotic medication and other dopamine receptor blocking agents.
- Olfactory reference disorder was added as a named example under other specified obsessive-compulsive and related disorder.

Neurodevelopmental disorders

Symptoms begin in the developmental period; gather reports from more than one setting and rule out hearing, vision, sleep and trauma.

Intellectual developmental disorder

Deficits in reasoning, problem solving, learning and judgment, confirmed by clinical assessment and testing, plus adaptive deficits in conceptual, social or practical skills, beginning in development.

Time: Onset in the developmental period. IQ usually about 2 standard deviations below the mean (roughly 70, plus or minus 5). Global developmental delay is used under age 5 when testing is not yet reliable.

vs. Autism spectrum disorder: Autism: social communication falls below the client's overall developmental level, with repetitive behaviors.

vs. Specific learning disorder: Learning disorder: trouble with specific academic skills despite typical intellectual and adaptive functioning.

Trap: *Severity (mild to profound) is set by adaptive functioning, not IQ alone. Test in the client's language with culturally fair measures.*

Autism spectrum disorder

Lasting deficits in social communication (all three areas required) plus at least 2 of 4 kinds of restricted, repetitive behavior, starting in early development.

Time: Early development; may not show fully until social demands exceed capacity. Severity levels 1 to 3 rated separately for each domain.

vs. Reactive attachment disorder: RAD: withdrawn behavior follows severe neglect, without the restricted, repetitive behavior pattern.

vs. Intellectual developmental disorder: IDD alone: social communication matches the client's overall developmental level.

Trap: *Social communication deficits without restricted, repetitive behaviors point to social (pragmatic) communication disorder, not autism.*

ADHD

Inattention and/or hyperactivity-impulsivity beyond developmental level, with several symptoms in two or more settings, interfering with school, work or social life.

Time: 6 or more symptoms (5 at age 17 and older) for 6 months; several present before age 12.

vs. Bipolar I disorder: ADHD is chronic and steady; bipolar comes in distinct episodes that differ from baseline.

vs. Disruptive mood dysregulation disorder: DMDD: persistent angry or irritable mood between severe outbursts, not just impulsivity.

Trap: *Symptoms in only one setting (home only) do not meet criteria; gather teacher and parent ratings.*

Tourette's disorder

Multiple motor tics and at least one vocal tic, not necessarily at the same time; tics may wax and wane in frequency.

Time: More than 1 year since the first tic; onset before age 18.

vs. Persistent motor or vocal tic disorder: Persistent tic disorder: motor or vocal tics, never both, for more than 1 year.

vs. Provisional tic disorder: Provisional: tics present for less than 1 year since the first tic.

Trap: *Coprolalia (involuntary swearing) occurs in only a minority and is not required. ADHD and OCD often co-occur.*

Schizophrenia spectrum and other psychotic disorders

Climb the duration ladder (1 day, 1 month, 6 months), ask whether psychosis ever occurs without a mood episode, and rule out substances and medical causes first.

Schizophrenia

Two or more of delusions, hallucinations, disorganized speech, disorganized or catatonic behavior, or negative symptoms (at least one of the first three), with a decline in functioning.

Time: 6 months or more of continuous signs, including at least 1 month of active-phase symptoms (less if successfully treated); prodromal and residual phases count.

vs. Schizophreniform disorder: Same symptoms, but 1 month to under 6 months; functional decline not required.

vs. Schizoaffective disorder: Schizoaffective: mood episodes fill most of the illness, plus 2 weeks of psychosis with no mood episode.

vs. Substance/medication-induced psychotic disorder: Substance-induced: psychosis starts with intoxication or withdrawal and clears within about a month of abstinence.

Trap: *Clinician bias leads to schizophrenia diagnoses in Black clients more often; weigh mood symptoms and realistic mistrust before calling suspicion psychotic.*

Schizophreniform disorder

Schizophrenia-type symptoms with a shorter course; a decline in daily functioning is not required.

Time: 1 month to under 6 months (active phase may be shorter if successfully treated). Label it 'provisional' while the client is still ill and 6 months have not passed.

vs. Brief psychotic disorder: Brief psychotic: under 1 month, with full return to prior functioning.

vs. Schizophrenia: Schizophrenia: 6 months or more of continuous signs, with functional decline.

Trap: *A client 4 months into symptoms and still ill gets schizophreniform disorder (provisional), not schizophrenia.*

Brief psychotic disorder

Sudden delusions, hallucinations or disorganized speech that resolve completely, often after a major stressor or after childbirth.

Time: At least 1 day and under 1 month, with full return to prior functioning.

vs. Schizophreniform disorder: Schizophreniform: symptoms last 1 month or longer.

vs. No mental disorder (normal or expected reaction): A culturally accepted experience, such as briefly hearing a deceased loved one, is not psychosis.

Trap: *Specify with or without marked stressors, or with onset during pregnancy or within 4 weeks after delivery. New psychosis after childbirth needs urgent evaluation.*

Schizoaffective disorder

One uninterrupted illness in which major mood episodes overlap with schizophrenia-type symptoms, and mood episodes are present for most of the total illness.

Time: At some point in the illness, delusions or hallucinations for 2 weeks or more with no major mood episode. Bipolar or depressive type.

vs. Bipolar I disorder: Bipolar I with psychotic features: psychosis occurs only during mood episodes.

vs. Major depressive disorder: MDD with psychotic features: psychosis appears only during depressive episodes.

vs. Schizophrenia: Schizophrenia: mood episodes, if any, fill only a small part of the illness.

Trap: *Psychosis 2 weeks or more without a mood episode rules out mood disorder with psychotic features; if mood episodes are brief overall, think schizophrenia.*

Delusional disorder

One or more delusions with behavior otherwise not odd; hallucinations, if present, are not prominent and fit the delusional theme.

Time: 1 month or more. Types: persecutory, jealous, erotomanic, grandiose, somatic, mixed.

vs. Schizophrenia: Schizophrenia: two or more core symptoms; delusional disorder has delusions with functioning largely intact.

vs. Paranoid personality disorder: Paranoid PD: lifelong distrust without a fixed false belief.

vs. Obsessive-compulsive disorder: OCD with absent insight stays OCD; the beliefs center on obsessions and rituals.

Trap: *Do not call a belief shared by the client's culture or faith community a delusion.*

Substance/medication-induced psychotic disorder

Delusions or hallucinations that start during or soon after intoxication, withdrawal or a medication, and are severe enough to need clinical attention.

Time: Suspect an independent psychotic disorder if psychosis began before use or persists about 1 month or more after withdrawal or severe intoxication ends.

vs. Schizophrenia: Schizophrenia: psychosis began before use or persists about 1 month after intoxication or withdrawal ends.

vs. Stimulant intoxication: Intoxication alone: expected drug effects, with reality testing intact.

vs. Delirium: Delirium: attention and awareness are disturbed and fluctuate.

Trap: *Methamphetamine, cannabis, hallucinogens, steroids and alcohol withdrawal are classic causes; ask about use before assuming schizophrenia.*

Psychotic disorder due to another medical condition

Prominent hallucinations or delusions that are the direct physical effect of a medical condition, such as epilepsy, a brain tumor, lupus or autoimmune encephalitis.

vs. Delirium: Delirium: psychosis occurs with fluctuating attention and awareness.

vs. Neurocognitive disorder with Lewy bodies: Lewy bodies: visual hallucinations with fluctuating cognition and parkinsonism.

vs. Schizophrenia: Schizophrenia: no medical cause, and onset usually in the late teens to twenties.

Trap: *First psychosis after about 40, or visual, tactile or smell hallucinations, suggest a medical or substance cause; refer for evaluation first.*

Bipolar and related disorders

Count the days and look for impairment or psychosis: hypomania 4 days, mania 1 week (any length if hospitalized); always screen for past highs before antidepressants.

Bipolar I disorder

At least one lifetime manic episode: elevated, expansive or irritable mood plus increased energy, with marked impairment, hospitalization or psychosis. Depression is common but not required.

Time: Manic episode: 1 week, most of the day, nearly every day, or any length if hospitalized; 3 or more symptoms (4 if mood is only irritable). Typical onset around 18.

vs. Bipolar II disorder: Bipolar II: hypomania only, never a full manic episode, plus at least one major depression.

vs. Borderline personality disorder: Borderline: mood shifts last hours and react to interpersonal stress; mania lasts a week or more.

vs. Schizoaffective disorder: Schizoaffective: psychosis also occurs for 2 weeks or more without any mood episode.

Trap: *Psychosis during elevated mood means mania, never hypomania. A full manic episode that outlasts an antidepressant's effect still counts.*

Bipolar II disorder

At least one hypomanic episode and at least one major depressive episode, with no lifetime mania. Depression usually drives the impairment; it is not a milder bipolar I.

Time: Hypomanic episode: 4 consecutive days or more, a change others notice, no marked impairment, no psychosis, no hospitalization.

vs. Major depressive disorder: MDD: no past hypomanic or manic episode. Always ask about past highs.

vs. Cyclothymic disorder: Cyclothymia: highs and lows never meet full hypomanic or major depressive episode criteria.

vs. Bipolar I disorder: Bipolar I: at least one manic episode, marked by a week, hospitalization or psychosis.

Trap: *Three days of elevated mood is not hypomania; it needs 4. An antidepressant alone can trigger a switch, so refer to the prescriber.*

Cyclothymic disorder

Many periods of hypomanic symptoms and many periods of depressive symptoms, none meeting full episode criteria, present at least half the time.

Time: 2 years in adults (1 year in youth); never symptom-free for more than 2 months.

vs. Persistent depressive disorder: PDD: chronic low mood with no hypomanic-type periods.

vs. Bipolar II disorder: Bipolar II: at least one full hypomanic and one full major depressive episode.

Trap: *Once a full manic, hypomanic or major depressive episode occurs, the picture is no longer cyclothymia.*

Substance/medication-induced bipolar disorder

Elevated, expansive or irritable mood that develops during or soon after use of, or withdrawal from, a substance or medication, such as stimulants or corticosteroids.

vs. Bipolar I disorder: A full mania that persists beyond an antidepressant's physiological effect counts as bipolar I.

vs. Stimulant intoxication: Intoxication alone: short-lived expected drug effects without a mood syndrome needing separate attention.

Trap: *Elevated mood only while using cocaine or high-dose steroids points here, not to bipolar I. One or two symptoms after an antidepressant are not enough.*

Depressive disorders

Count 5 of 9 symptoms for 2 weeks, ask about any past mania or hypomania, and rule out substances, medications and medical illness before calling it depression.

Major depressive disorder

Depressed mood or loss of interest plus other symptoms (sleep, guilt, energy, concentration, appetite, psychomotor change, suicidal thoughts), 5 of 9 in all, with no past mania or hypomania.

Time: Same 2-week period; in youth, mood may be irritable. Peripartum onset: during pregnancy or within 4 weeks after delivery. Recurrent: at least 2 months in a row between episodes without full criteria.

vs. Adjustment disorder: If full MDD criteria are met after a stressor, diagnose MDD, not adjustment disorder.

vs. Prolonged grief disorder: PGD centers on yearning for the deceased; depression brings pervasive low mood and worthlessness.

vs. Bipolar II disorder: Bipolar II: a past hypomanic episode, even one the client enjoyed.

Trap: *The bereavement exclusion is gone: grief comes in waves with self-esteem intact, but diagnose MDD if full criteria are met.*

Persistent depressive disorder

Chronic depressed mood most of the day, more days than not, plus 2 or more symptoms such as poor appetite or sleep, low energy, low self-esteem or hopelessness.

Time: 2 years in adults (1 year in youth, when mood may be irritable); never symptom-free for more than 2 months. Specify early (before 21) or late onset.

vs. Major depressive disorder: Episode criteria met for all 2 years: specify with persistent major depressive episode; diagnose MDD too.

vs. Cyclothymic disorder: Cyclothymia includes hypomanic-type periods; PDD never has highs.

Trap: *'I've always been like this' suggests PDD. If major depressive episode criteria are met during PDD, also diagnose MDD and add the PDD pattern specifier.*

Disruptive mood dysregulation disorder

Severe temper outbursts far out of proportion, about 3 or more times a week, with persistently irritable or angry mood between them, in at least 2 settings.

Time: 12 months or more with no 3-month break; first diagnosed between ages 6 and 18; onset before 10.

vs. Bipolar I disorder: Bipolar: distinct episodes of elevated mood. A manic-type period lasting more than 1 day rules out DMDD.

vs. Oppositional defiant disorder: If criteria for both are met, diagnose DMDD only.

vs. Intermittent explosive disorder: IED: mood is normal between outbursts.

Trap: *DMDD was added partly to curb overdiagnosis of pediatric bipolar disorder; chronic irritability is not episodic mania.*

Premenstrual dysphoric disorder

Most cycles in the past year: 5 or more symptoms, at least one being mood swings, irritability, low mood or anxiety, in the week before menses, easing after onset.

Time: Symptoms improve within a few days after menses begin and are minimal or gone the week after. Confirm with daily ratings over at least 2 cycles; until then, mark it provisional.

vs. Major depressive disorder: MDD: symptoms persist across the whole cycle; premenstrual worsening of MDD is not PMDD.

vs. No mental disorder (normal or expected reaction): Common premenstrual discomfort without marked distress or impairment is not PMDD.

Trap: *Do not diagnose from memory of past cycles; prospective daily ratings across 2 cycles confirm it.*

Substance/medication-induced depressive disorder

Prominent depressed mood or loss of interest that develops during or soon after intoxication, withdrawal or a medication such as interferon, corticosteroids or isotretinoin.

vs. Major depressive disorder: MDD: depression began before use or persists about 1 month after withdrawal or intoxication ends.

vs. Depressive disorder due to another medical condition: Medical cause: a physical illness, not a drug, explains the mood change.

Trap: *Ask about alcohol, drugs and new prescriptions before diagnosing MDD. A stimulant crash can bring severe depression and suicidal thoughts.*

Depressive disorder due to another medical condition

Depressed mood that is the direct physical effect of a medical condition such as hypothyroidism, stroke, Parkinson's disease, Cushing's syndrome or pancreatic cancer.

vs. Major depressive disorder: MDD: no medical condition explains the mood change through a physical mechanism.

vs. Adjustment disorder: Adjustment: an emotional reaction to the stress of being ill, not a physical effect.

Trap: *New depression with fatigue, weight change or neurological signs, especially later in life, calls for a medical evaluation first.*

Anxiety disorders

Name the feared target: diffuse worry (GAD), unexpected attacks (panic), being judged (social anxiety) or separation; most adult thresholds are 6 months.

Separation anxiety disorder

Fear of separation from attachment figures beyond what fits the client's development, such as worry they will be harmed or refusing to leave home or sleep alone.

Time: 4 weeks or more in children and teens; typically 6 months or more in adults.

vs. Agoraphobia: Agoraphobia: fear that escape or help will be unavailable, not fear of losing attachment figures.

vs. Social anxiety disorder: Social anxiety: school refusal driven by fear of peer judgment, not separation.

Trap: *It can be diagnosed in adults, such as an adult who cannot tolerate a partner being away.*

Selective mutism

Consistent failure to speak in specific social situations where speech is expected, such as school, while speaking normally in others, such as at home.

Time: At least 1 month, not limited to the first month of school. Onset usually before age 5.

vs. Social anxiety disorder: Social anxiety often co-occurs; it is fear of judgment, not a setting-specific failure to speak.

vs. Autism spectrum disorder: Autism: social communication deficits in all settings, not speech limited to certain settings.

Trap: *A recent immigrant child silent at school while still learning English does not have selective mutism; assess language first.*

Specific phobia

Marked, immediate fear of a specific object or situation, such as animals, heights, flying, needles or blood, out of proportion to actual danger and avoided.

Time: Typically 6 months or more. Types: animal, natural environment, blood-injection-injury, situational, other.

vs. Agoraphobia: Agoraphobia: fear of 2 or more situation types because escape or help may be hard.

vs. Panic disorder: Panic disorder: attacks come unexpectedly, not only when facing one feared cue.

Trap: *Blood-injection-injury type can cause fainting from a drop in heart rate and blood pressure. Exposure therapy is first-line.*

Social anxiety disorder

Marked fear of social or performance situations where others may judge, with fear of embarrassment; situations are avoided or endured with intense distress.

Time: 6 months or more. In children, the anxiety must occur with peers, not only with adults.

vs. Panic disorder: Panic attacks only when speaking or performing point to social anxiety, not panic disorder.

vs. Avoidant personality disorder: Avoidant PD: a broader, lifelong pattern of feeling inadequate across most relationships.

Trap: *Taijin kyofusho centers on fear of offending others, not of embarrassing oneself; assess the cultural framing.*

Panic disorder

Recurrent unexpected panic attacks (a surge of intense fear peaking within minutes), followed by worry about more attacks or a change in behavior to avoid them.

Time: At least 1 month of worry or behavior change after an attack. An attack has 4 or more of 13 symptoms.

vs. Agoraphobia: Agoraphobia: fear of 2 or more situations where escape or help is hard; diagnosed separately.

vs. Generalized anxiety disorder: GAD: ongoing diffuse worry, not discrete attacks.

vs. Anxiety disorder due to another medical condition: Medical cause such as hyperthyroidism, arrhythmia or pheochromocytoma explains the attacks.

Trap: *Rule out thyroid, heart rhythm, caffeine and stimulants first. Ataque de nervios usually follows a stressor, so it is not an unexpected attack.*

Agoraphobia

Marked fear or avoidance of 2 or more of 5 situations (public transport, open spaces, enclosed places, crowds or lines, outside home alone) because escape or help may be unavailable.

Time: 6 months or more. Diagnosed with or without panic disorder; if both are met, give both.

vs. Panic disorder: Panic disorder: unexpected attacks plus worry about them; agoraphobic avoidance is diagnosed separately.

vs. Specific phobia: Specific phobia: fear limited to one situation type, such as flying.

vs. Social anxiety disorder: Social anxiety: avoids crowds for fear of judgment, not of being trapped.

Trap: *An older adult afraid to go out may fear falling; weigh medical causes and realistic risk before calling it agoraphobia.*

Generalized anxiety disorder

Excessive, hard-to-control worry about many areas of life, with 3 or more of restlessness, fatigue, poor concentration, irritability, muscle tension or poor sleep (1 in children).

Time: More days than not for 6 months or more.

vs. Obsessive-compulsive disorder: OCD: intrusive unwanted thoughts with rituals, not everyday real-life worries.

vs. Illness anxiety disorder: Illness anxiety: worry is fixed on having a serious illness.

vs. Adjustment disorder: Adjustment with anxiety: tied to one stressor, starts within 3 months, below GAD criteria.

Trap: *Caffeine, stimulants, hyperthyroidism, and alcohol or benzodiazepine withdrawal can all look like GAD.*

Obsessive-compulsive and related disorders

Obsessions are unwanted (ego-dystonic); poor or absent insight does not turn OCD into psychosis.

Obsessive-compulsive disorder

Intrusive, unwanted thoughts, urges or images and/or repetitive acts done to reduce distress or prevent a feared event, which are excessive or not realistically connected.

Time: Time-consuming (more than 1 hour a day) or causes distress or impairment.

vs. Obsessive-compulsive personality disorder: OCPD: ego-syntonic perfectionism and control, with no true obsessions or rituals.

vs. Generalized anxiety disorder: GAD: worries about real-life matters, with no neutralizing rituals.

vs. Body dysmorphic disorder: BDD: preoccupation centers on a perceived flaw in appearance that others see as slight.

Trap: *Absent insight or delusional beliefs is an OCD specifier, not psychosis. First-line care is ERP and SSRIs.*

Body dysmorphic disorder

Preoccupation with one or more perceived flaws in appearance that others cannot see or see as slight, with repetitive acts such as mirror checking, grooming, reassurance seeking or comparing.

Time: Specify 'with muscle dysmorphia' and level of insight (good or fair, poor, or absent/delusional).

vs. Anorexia nervosa: If the concern is only body fat or weight in an eating disorder, diagnose the eating disorder.

vs. Obsessive-compulsive disorder: OCD: obsessions about contamination, harm or symmetry, not appearance.

vs. Delusional disorder: BDD with absent insight stays BDD, not delusional disorder.

Trap: *Cosmetic procedures rarely help and suicide risk is high; assess safety and consider CBT or an SSRI.*

Hoarding disorder

Persistent difficulty discarding possessions regardless of value, with distress at parting with them, leaving living areas so cluttered they cannot be used as intended.

vs. Obsessive-compulsive disorder: OCD: saving is driven by a typical obsession, such as fear of contamination or harm.

vs. Major neurocognitive disorder: Neurocognitive: accumulation follows cognitive decline or brain injury, not a long-standing saving pattern.

Trap: *Specify excessive acquisition and level of insight. In older adults, assess fire risk, falls and self-neglect.*

Trauma- and stressor-related disorders

Sort by timing and stressor: acute stress 3 days to 1 month, PTSD over 1 month, adjustment within 3 months of any stressor, prolonged grief 12 months after a death.

Reactive attachment disorder

A consistent pattern of withdrawn, inhibited behavior toward caregivers: rarely seeks or responds to comfort when upset, with limited positive emotion and unexplained irritability, sadness or fear.

Time: Follows extremely insufficient care (neglect, frequent caregiver changes, institutional rearing). Developmental age at least 9 months; evident before age 5.

vs. Autism spectrum disorder: Autism: restricted, repetitive behaviors, no neglect history needed, and selective attachment is usually present.

vs. Disinhibited social engagement disorder: DSED: overly friendly with strangers rather than withdrawn from caregivers.

Trap: *Both RAD and DSED require a history of severely insufficient care; RAD usually improves once a stable, responsive caregiver is available.*

Disinhibited social engagement disorder

Actively approaches and engages unfamiliar adults with little hesitation: overly familiar, does not check back with caregivers, and is willing to go off with strangers.

Time: Follows extremely insufficient care. Developmental age at least 9 months.

vs. ADHD: ADHD: general impulsivity, not socially disinhibited behavior tied to a history of neglect.

vs. Reactive attachment disorder: RAD: withdrawn and inhibited toward caregivers rather than indiscriminately social.

Trap: *Signs can persist after placement with a stable, loving family, even once the child attaches to new caregivers.*

Posttraumatic stress disorder

After actual or threatened death, serious injury or sexual violence: intrusion, avoidance, negative changes in thinking and mood, and heightened arousal or reactivity.

Time: More than 1 month. 'With delayed expression' if full criteria are not met until 6 months or more after the event.

vs. Acute stress disorder: Acute stress disorder: 3 days to 1 month after the trauma.

vs. Adjustment disorder: Adjustment: any stressor (divorce, job loss), or trauma without full PTSD criteria.

Trap: *Exposure through TV or social media does not count unless it is part of the job. Separate criteria apply at age 6 and under.*

Acute stress disorder

Nine or more symptoms of intrusion, negative mood, dissociation, avoidance or arousal after the same kind of traumatic exposure required for PTSD.

Time: 3 days to 1 month after the trauma.

vs. Posttraumatic stress disorder: Symptoms that last beyond 1 month and meet criteria become PTSD.

vs. Adjustment disorder: Adjustment: the stressor need not be traumatic, and symptoms fall below acute stress criteria.

Trap: *Dissociation is not required: any 9 of the 14 symptoms across the five categories count.*

Adjustment disorder

Emotional or behavioral symptoms out of proportion to an identifiable stressor, or impairing, when no other disorder's criteria are met.

Time: Onset within 3 months of the stressor; resolves within 6 months after the stressor or its consequences end.

vs. Major depressive disorder: Full MDD criteria after a stressor: diagnose MDD.

vs. No mental disorder (normal or expected reaction): Normal stress: the reaction is proportionate and does not impair functioning.

vs. Prolonged grief disorder: A grief reaction beyond norms that does not meet PGD, such as before 12 months, can be adjustment disorder.

Trap: *Adjustment disorder is a residual category; check MDD, PTSD and GAD criteria first.*

Prolonged grief disorder

Intense yearning for or preoccupation with the deceased nearly every day, plus 3 or more symptoms such as identity disruption, disbelief, numbness or meaninglessness, beyond cultural norms.

Time: Death at least 12 months ago in adults (6 months in children and teens); symptoms for at least the last month.

vs. Major depressive disorder: MDD: pervasive low mood and worthlessness, not centered on the person who died.

vs. Posttraumatic stress disorder: PTSD: fear and avoidance tied to a traumatic event, not yearning.

vs. No mental disorder (normal or expected reaction): Normal grief: waves of sadness, self-esteem intact, within cultural norms.

Trap: *An adult grieving intensely at 7 months cannot have PGD; consider normal grief, MDD, or adjustment disorder if out of proportion. New in DSM-5-TR.*

Dissociative disorders

Reality testing stays intact; rule out seizures, substances and head injury, and respect cultural and religious practices.

Dissociative identity disorder

Two or more distinct personality states (described as possession in some cultures) with breaks in sense of self, plus recurrent memory gaps beyond ordinary forgetting.

vs. Dissociative amnesia: Amnesia: memory gaps without distinct identity states.

vs. Borderline personality disorder: Borderline: unstable self-image and brief stress-related dissociation, without distinct personality states.

Trap: *Not diagnosed when part of accepted cultural or religious practice, or imaginary play in children. Strongly linked to severe early abuse.*

Depersonalization/derealization disorder

Persistent or recurrent feelings of detachment from oneself or of the world seeming unreal, while the client knows the feeling is not real.

vs. Schizophrenia: Psychosis: reality testing is lost.

vs. Panic disorder: Panic: the unreality occurs only during attacks.

Trap: *Unreality that occurs only during panic attacks, drug use or PTSD does not count as a separate disorder.*

Somatic symptom and related disorders (plus malingering)

Ask two questions: is it intentional, and is there an external reward? No and no: somatic; yes and no: factitious; yes and yes: malingering.

Somatic symptom disorder

One or more distressing body symptoms with excessive thoughts, anxiety, or time and energy spent on them; symptoms may or may not have a medical explanation.

Time: Persistent, typically more than 6 months.

vs. Illness anxiety disorder: Illness anxiety: fear of having a serious illness, with symptoms absent or mild.

vs. Factitious disorder imposed on self: Factitious: symptoms are deliberately faked or induced.

Trap: *A real medical diagnosis does not rule it out; the excessive response to the symptom is the focus.*

Functional neurological symptom disorder

Weakness, paralysis, non-epileptic seizures, numbness, blindness or gait change that clinical findings show is incompatible with known neurological disease; not intentional.

Time: Acute (under 6 months) or persistent (6 months or more).

vs. Malingering: Malingering: symptoms are faked on purpose for an external reward.

vs. Factitious disorder imposed on self: Factitious: deliberate deception to be seen as ill.

Trap: *A psychological stressor is not required, and calm indifference is not diagnostic. Never treat it as faking.*

Factitious disorder imposed on self

Deliberately falsifying or inducing illness or injury and presenting as sick, with no obvious external reward; the motive is the sick role.

vs. Malingering: Malingering: a clear external incentive such as money, drugs or avoiding jail.

vs. Factitious disorder imposed on another: Imposed on another: illness is faked or induced in a child, elder or pet.

Trap: *When imposed on another, the perpetrator (often a caregiver) receives the diagnosis, and abuse reporting duties apply.*

Malingering

Intentionally faking or exaggerating symptoms for an external reward, such as money, drugs, or avoiding work, military duty or jail. A Z code, not a mental disorder.

vs. Factitious disorder imposed on self: Factitious: deception without an obvious external reward.

vs. Somatic symptom disorder: Somatic symptom: symptoms are real to the client and not produced on purpose.

Trap: *Suspect it in legal or compensation settings, or when reported and observed function differ. Assess; do not accuse.*

Feeding and eating disorders

Ask about body image and compensatory behavior; anorexia severity uses BMI, while bulimia and binge-eating severity use episodes per week.

Anorexia nervosa

Restricted intake leading to significantly low body weight, intense fear of gaining weight or behavior that blocks gain, and disturbed body image or self-worth tied to weight.

Time: Type (restricting or binge-eating/purging) set by the past 3 months. Adult BMI severity: mild 17 or more, moderate 16-16.99, severe 15-15.99, extreme under 15; youth use BMI percentiles.

vs. Avoidant/restrictive food intake disorder: ARFID: no fear of weight gain and no body-image disturbance.

vs. Bulimia nervosa: Bulimia: weight is normal or above; anorexia requires significantly low weight.

Trap: *Amenorrhea is no longer required. Among the highest mortality of psychiatric disorders; refeeding needs medical monitoring.*

Bulimia nervosa

Recurrent binges (a large amount with loss of control) and recurrent compensatory acts such as vomiting, laxatives, fasting or overexercise, with self-worth tied to shape or weight.

Time: Both at least once a week for 3 months. Severity by compensatory episodes a week: mild 1-3, moderate 4-7, severe 8-13, extreme 14 or more.

vs. Binge-eating disorder: Binge-eating disorder: no regular compensatory behavior.

vs. Anorexia nervosa: Binge-purging only during anorexia: diagnose anorexia, binge-eating/purging type.

Trap: *Weight is often normal, so look for dental erosion, knuckle calluses, swollen parotid glands and low potassium (cardiac risk).*

Avoidant/restrictive food intake disorder

Restricted eating causing weight loss, nutritional deficiency, supplement dependence or social impairment, due to low interest, sensory dislikes or fear of choking or vomiting.

vs. Anorexia nervosa: Anorexia: restriction driven by fear of weight gain and body-image disturbance.

vs. Autism spectrum disorder: Picky eating within autism gets an added ARFID diagnosis only when it needs separate clinical attention.

Elimination disorders

Rule out medical causes first, and ask about stressors when a child who had been continent starts having accidents.

Enuresis

Repeated urination into bed or clothes, involuntary or intentional, not due to a substance or medical condition such as diabetes, a UTI or seizures.

Time: Age 5 or older; twice a week for 3 months in a row, or with significant distress or impairment.

vs. Encopresis: Encopresis: feces, not urine; minimum age 4.

Trap: *Secondary enuresis (after a period of established dryness) suggests a stressor such as a new sibling, divorce or abuse; rule out medical causes.*

Encopresis

Repeated passage of feces in inappropriate places, most often from constipation with overflow; not due to a substance or medical condition.

Time: Age 4 or older; at least once a month for 3 months.

vs. Enuresis: Enuresis: urine, not feces; minimum age 5.

Trap: *Refer for a medical check for constipation before starting behavioral treatment.*

Sleep-wake disorders

Sleep problems can mimic depression, ADHD and cognitive decline; refer for a sleep study when snoring or gasping suggests apnea.

Insomnia disorder

Trouble falling asleep, staying asleep or waking too early, with daytime distress or impairment, despite adequate opportunity to sleep.

Time: 3 nights a week or more, for 3 months or more.

vs. Narcolepsy: Narcolepsy: irresistible daytime sleep episodes, with cataplexy or related findings.

Trap: *CBT for insomnia is first-line, before sleep medication.*

Gender dysphoria

Use DSM-5-TR terms: experienced gender, gender-affirming, assigned male or female at birth.

Gender dysphoria

Marked incongruence between experienced and assigned gender with clinically significant distress or impairment; gender diversity itself is not a disorder.

Time: 6 months or more.

vs. No mental disorder (normal or expected reaction): Gender-diverse identity without distress or impairment needs no diagnosis.

Disruptive, impulse-control and conduct disorders

Match the pattern: defiance with authority (ODD), violating others' rights (conduct), episodic outbursts with calm between (IED), constant irritability (DMDD).

Oppositional defiant disorder

Angry or irritable mood, argumentative or defiant behavior, or vindictiveness, shown with at least one person who is not a sibling.

Time: 4 or more symptoms for 6 months; at least weekly from age 5, most days under age 5.

vs. Conduct disorder: Conduct: serious violations of others' rights, such as aggression, theft or property destruction.

vs. Disruptive mood dysregulation disorder: If DMDD criteria are met, diagnose DMDD, not ODD.

Trap: *Severity counts settings: mild 1, moderate 2, severe 3 or more.*

Conduct disorder

Repeated violation of others' rights or major age rules: aggression to people or animals, property destruction, deceit or theft, or serious rule violations.

Time: 3 or more behaviors in the past 12 months, at least 1 in the past 6; childhood-onset type if one began before age 10.

vs. Antisocial personality disorder: ASPD: age 18 or older, with conduct disorder that began before 15.

vs. Oppositional defiant disorder: ODD: defiance and irritability without serious violations of others' rights.

Trap: *Specify 'with limited prosocial emotions' for callous, unemotional traits. Childhood onset predicts a worse course.*

Intermittent explosive disorder

Recurrent impulsive aggressive outbursts, far out of proportion to the trigger, not planned or aimed at a goal, with normal mood between them.

Time: Age 6 or older. Verbal or non-damaging outbursts about twice a week for 3 months, or 3 damaging outbursts within 12 months.

vs. Disruptive mood dysregulation disorder: DMDD: persistent irritability between outbursts, onset before 10.

vs. Antisocial personality disorder: ASPD: aggression is often planned or used for gain, with little remorse.

Substance-related and addictive disorders

Count 2 or more of 11 criteria in 12 months and ask about last use; alcohol and sedative withdrawal can kill, while the opioid danger is overdose.

Alcohol use disorder

A problem drinking pattern with impaired control, social problems, risky use, or tolerance and withdrawal, meeting 2 or more of 11 criteria.

Time: Within 12 months. Mild 2-3, moderate 4-5, severe 6 or more. Early remission 3 to under 12 months; sustained 12 months or more (craving allowed).

vs. Alcohol intoxication: Intoxication is one current state; use disorder is a 12-month pattern.

vs. Alcohol withdrawal: Withdrawal is the current syndrome after stopping; it can occur with or without the use disorder.

Trap: *A positive AUDIT (8 or more) or CAGE (2 or more) flags risk but does not diagnose; confirm with the criteria.*

Alcohol withdrawal

After heavy, prolonged drinking stops or drops: sweating, fast pulse, hand tremor, insomnia, nausea, anxiety, agitation, brief hallucinations and possible seizures.

Time: Typical from last drink: tremor 6-12 hours, hallucinosis 12-24 hours, seizures 24-48 hours, delirium tremens 48-96 hours.

vs. Opioid withdrawal: Opioid withdrawal is miserable but rarely fatal; alcohol withdrawal can cause seizures and death.

vs. Delirium: Delirium tremens is withdrawal delirium: confusion, fever and fluctuating attention; a medical emergency.

Trap: *The first step is medically supervised withdrawal management (benzodiazepines, thiamine), not outpatient counseling. CIWA-Ar guides dosing.*

Opioid use disorder

Problem opioid use meeting 2 or more of 11 criteria, such as craving, failed attempts to cut down, role failure and use despite harm.

Time: Within 12 months; severity and remission as for alcohol. Specify 'on maintenance therapy' with methadone, buprenorphine or naltrexone.

vs. Opioid withdrawal: Withdrawal is the current syndrome after stopping; use disorder is the 12-month pattern.

vs. No mental disorder (normal or expected reaction): Tolerance and withdrawal from opioids taken as prescribed do not count toward the disorder.

Trap: *Overdose risk spikes after detox or jail because tolerance drops; provide naloxone education.*

Opioid withdrawal

After stopping or cutting opioids: low mood, nausea, muscle aches, tearing, runny nose, dilated pupils, goosebumps, sweating, diarrhea, yawning, fever and insomnia.

vs. Alcohol withdrawal: Alcohol withdrawal: tremor, seizure risk and possible delirium; it can be fatal.

vs. Stimulant intoxication: Both dilate pupils; stimulant intoxication brings euphoria and hypervigilance, not aches and diarrhea.

Trap: *Pinpoint pupils mean intoxication; dilated pupils mean withdrawal. Overdose (pinpoint pupils, unresponsive, slow breathing): call 911, give naloxone.*

Stimulant intoxication

Recent cocaine or amphetamine-type use with euphoria or agitation, hypervigilance, grandiosity, fast heart rate, dilated pupils, high blood pressure, and sometimes paranoia or chest pain.

vs. Bipolar I disorder: Mania lasts a week or more and is not explained by drug use.

vs. Panic disorder: Panic: attacks occur without any stimulant use.

Trap: *The crash afterward brings fatigue, hunger, vivid dreams and depression; check suicide risk.*

Sedative, hypnotic or anxiolytic withdrawal

After stopping or cutting long-term benzodiazepine or similar use: autonomic hyperactivity, tremor, insomnia, anxiety, hallucinations and possible seizures, much like alcohol withdrawal.

vs. Generalized anxiety disorder: Rebound anxiety after stopping a benzodiazepine is withdrawal, not new GAD.

vs. Opioid withdrawal: Opioid withdrawal is rarely dangerous; sedative withdrawal can cause seizures and delirium.

Trap: *Never advise stopping abruptly; refer for a medically supervised taper. Tolerance from prescribed use alone is not a use disorder.*

Gambling disorder

Persistent problem gambling with signs such as chasing losses, lying about gambling, needing bigger bets, failed attempts to stop and relying on others for money.

Time: 4 or more of 9 criteria within 12 months.

vs. Bipolar I disorder: If gambling occurs only during manic episodes, do not add gambling disorder.

Trap: *The only behavioral addiction in Section II; internet gaming disorder is a Section III condition for further study.*

Neurocognitive disorders

Sort by onset and attention: acute and fluctuating means delirium (an emergency); gradual with intact attention means NCD; independence in daily tasks separates major from mild.

Delirium

Acute disturbance in attention and awareness that fluctuates over the day, plus another cognitive change, caused by a medical condition, substance, withdrawal or medication.

Time: Develops over hours to days; often worse at night. Hyperactive, hypoactive or mixed.

vs. Major neurocognitive disorder: Dementia: gradual onset over months to years; attention intact until late.

vs. Major depressive disorder: Depression: attention intact. Quiet hypoactive delirium is often mistaken for depression.

Trap: *Next step is urgent medical evaluation. Common causes in older adults: UTI, pneumonia, dehydration, new medications, surgery, alcohol withdrawal.*

Major neurocognitive disorder

Significant decline from prior level in one or more cognitive domains, shown on testing, that interferes with independence; at minimum needs help with bills or medications.

Time: Usually gradual. Specify cause: Alzheimer's (memory first), vascular (stepwise, tied to strokes), frontotemporal (behavior or language, onset often 50s-60s). Severity: mild, moderate, severe.

vs. Mild neurocognitive disorder: Mild: independence preserved, perhaps with extra effort or reminders.

vs. Delirium: Delirium: acute onset, fluctuating attention, a medical cause.

vs. Major depressive disorder: Depression: datable onset, 'I don't know' answers, stresses deficits; improves with treatment.

Trap: *Rule out reversible causes first: delirium, depression, medications, B12 deficiency, hypothyroidism, normal pressure hydrocephalus.*

Mild neurocognitive disorder

Modest decline from prior level in one or more cognitive domains, with everyday independence preserved, though tasks take more effort, strategies or reminders.

vs. Major neurocognitive disorder: Major: needs help with complex daily tasks such as bills or medications.

vs. No mental disorder (normal or expected reaction): Normal aging: slower recall without a measurable decline on standardized testing.

Trap: *An older adult who still pays bills using reminders has mild, not major, NCD. The MoCA detects it better than the MMSE.*

Neurocognitive disorder with Lewy bodies

Cognitive decline with fluctuating attention and alertness, recurrent detailed visual hallucinations, and parkinsonism that begins after the cognitive decline; REM sleep behavior disorder is common.

Time: Gradual onset and progression.

vs. Delirium: Delirium: acute onset with a medical cause; fluctuations clear when the cause is treated.

vs. Schizophrenia: Schizophrenia: earlier onset, mostly auditory hallucinations, no fluctuating alertness or parkinsonism.

Trap: *Severe sensitivity to antipsychotics. Never assume new visual hallucinations in an older adult are schizophrenia.*

Personality disorders

Cluster A (odd): paranoid, schizoid, schizotypal; Cluster B (dramatic): antisocial, borderline, histrionic, narcissistic; Cluster C (anxious): avoidant, dependent, obsessive-compulsive. Under 18, traits must have lasted 1 year; antisocial PD never before 18.

Paranoid personality disorder

Pervasive distrust and suspicion; reads hidden threats into harmless remarks, bears grudges and is reluctant to confide in others.

Time: Enduring pattern from adolescence or early adulthood.

vs. Delusional disorder: Delusional disorder: a fixed false belief; paranoid PD is suspiciousness without delusions.

vs. Schizotypal personality disorder: Schizotypal: odd beliefs, magical thinking and eccentric behavior beyond suspicion.

Trap: *Weigh the client's real experiences of discrimination; realistic, culturally grounded mistrust is not paranoia.*

Schizoid personality disorder

Detached from relationships and indifferent to them, with a narrow range of emotion; prefers solitary activities and seems unmoved by praise or criticism.

Time: Enduring pattern from adolescence or early adulthood.

vs. Avoidant personality disorder: Avoidant: wants relationships but fears rejection; schizoid does not want them.

vs. Schizotypal personality disorder: Schizotypal: odd beliefs and perceptual distortions; schizoid has none.

Trap: *A client who longs for closeness but stays away is avoidant, not schizoid.*

Schizotypal personality disorder

Acute discomfort in close relationships, odd beliefs or magical thinking, ideas of reference and eccentric behavior; social anxiety does not ease with familiarity.

Time: Enduring pattern. Listed in both the psychotic and the personality chapters.

vs. Schizophrenia: Schizophrenia: full delusions and hallucinations; schizotypal ideas of reference fall short of delusion.

vs. Schizoid personality disorder: Schizoid: detached, but without odd beliefs or perceptual distortions.

Trap: *Do not label culturally shared beliefs, such as in spirits or rituals, as magical thinking.*

Antisocial personality disorder

Pervasive disregard for and violation of others' rights: deceit, impulsivity, aggression, irresponsibility and lack of remorse.

Time: Age 18 or older, with conduct disorder that began before age 15.

vs. Narcissistic personality disorder: Narcissistic: seeks admiration; antisocial seeks gain or power and lacks remorse.

vs. Conduct disorder: Under 18: diagnose conduct disorder, never antisocial PD.

Trap: *Antisocial acts that occur only during schizophrenia or bipolar episodes do not support this diagnosis.*

Borderline personality disorder

Unstable relationships, self-image and mood; impulsivity, frantic fear of abandonment, recurrent self-harm or suicidal behavior, emptiness, intense anger, brief stress-related paranoia or dissociation.

Time: 5 or more of 9 features; enduring pattern from adolescence or early adulthood.

vs. Bipolar I disorder: Bipolar: sustained mood episodes over days to weeks; borderline shifts take hours and follow interpersonal stress.

vs. Dependent personality disorder: Dependent: meets abandonment with appeasement; borderline meets it with rage and emptiness.

Trap: *DBT is first-line. Assess self-harm every time; do not dismiss it as 'attention seeking' or 'manipulative'.*

Histrionic personality disorder

Excessive emotionality and attention seeking; uncomfortable when not the center of attention, shallow shifting emotions, suggestible, and sees relationships as closer than they are.

Time: Enduring pattern from adolescence or early adulthood.

vs. Narcissistic personality disorder: Narcissistic: wants admiration for superiority; histrionic accepts any attention, even appearing fragile.

vs. Borderline personality disorder: Borderline: self-harm, emptiness and identity disturbance, which histrionic lacks.

Narcissistic personality disorder

Grandiosity, need for admiration and lack of empathy, with entitlement, envy and exploitation of others; self-esteem underneath is often fragile.

Time: Enduring pattern from adolescence or early adulthood.

vs. Antisocial personality disorder: Antisocial: seeks material gain, with deceit and lack of remorse, more than admiration.

vs. Bipolar I disorder: Mania: grandiosity comes in episodes with less need for sleep; narcissistic grandiosity is stable.

Avoidant personality disorder

Social inhibition, feelings of inadequacy and hypersensitivity to criticism; wants relationships but avoids them unless sure of being liked.

Time: Enduring pattern from adolescence or early adulthood.

vs. Schizoid personality disorder: Schizoid: does not want relationships at all.

vs. Social anxiety disorder: Heavy overlap; avoidant PD is more pervasive, rooted in a core sense of inadequacy.

Dependent personality disorder

Excessive need to be cared for; submissive and clinging, struggles with everyday decisions without reassurance, and urgently seeks a new relationship when one ends.

Time: Enduring pattern from adolescence or early adulthood.

vs. Borderline personality disorder: Borderline: reacts to abandonment with rage and emptiness, not appeasement.

vs. Avoidant personality disorder: Avoidant: withdraws from relationships for fear of rejection; dependent clings to them.

Trap: *Staying in an abusive relationship is not proof of dependent PD; assess safety, money and fear first.*

Obsessive-compulsive personality disorder

Preoccupation with order, perfectionism and control at the cost of flexibility; rigid, overworking, reluctant to delegate, miserly, and may keep worthless objects.

Time: Enduring pattern from adolescence or early adulthood.

vs. Obsessive-compulsive disorder: OCD: unwanted obsessions and rituals; OCPD traits feel right to the client (ego-syntonic).

vs. Hoarding disorder: Hoarding: clutter blocks use of living space and causes distress; diagnose both if extreme.

Trap: *OCD and OCPD can co-occur. A manager proud of rigid standards points to OCPD.*

Main source: American Psychiatric Association (2022). *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.). Entry-level sources are listed in the app.